

Recovery Facility Levels of Care

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Introduction

Recovery facilities provide professional skilled services that facilitate patient recovery and stability. Rehabilitation for post-acute care may be considered when a patient continues to need skilled healthcare services that can be managed in the recovery facility environment. When primarily rehabilitation services are needed, health professionals working in acute hospitals should recognize patients' needs for rehabilitation care and facilitate transition to appropriate rehabilitation care settings.(1)

This tool provides guidance in assessing clinical needs for admission and continuing stay for patients requiring recovery facility level of care. It focuses on the aspects that a hospital inpatient care manager may need to know to transition a patient to a less-intensive level of care. It is not intended to duplicate or replace the information found in General Recovery Care or Recovery Facility Care guidelines for a specific diagnosis or condition.

Overview of Skilled Nursing Facility Data Collection Systems

Medicare payment for skilled nursing facility services is through a health insurance prospective payment system (HIPPS). This system collects data to establish best practice, understand costs, identify cost-effective systems and procedures, and determine reimbursement for services. Reimbursement is independent of the site of care, but rates of payment may be adjusted for case-mix and geographic variations in wages and costs.(2) The data collection system to determine the HIPPS in skilled nursing facilities (SNF) is the Minimum Data Set (MDS). HIPPS uses 5-character codes; the first 4 positions represent the Patient Driven Payment Model (PDPM) case mix classification codes for each PDPM component (physical and occupational therapy, speech therapy, nursing, and non-therapy ancillary services) and the fifth position reflects an assessment indicator that indicates the reason and time frame for assessment completion (via the MDS).(3)

MDS 3.0 is an assessment tool to identify problems, strengths, and preferences to be addressed in an individualized care plan. It is divided into 2 sections, covering the patient's status in cognition, communication, hearing and vision, mood, behavior, physical functioning, continence, active diagnoses, health condition, nutritional status, oral status, skin conditions, medication, procedures, and discharge potential.[A] MDS 3.0 also has a section on patient participation in assessment and goal setting. In a SNF, the MDS is completed on admission and at ongoing intervals. The person completing the form may be a designated MDS coordinator, or someone from each functional area (eg, nursing, therapy, dietary) who may complete a corresponding section.

Patient-Driven Payment Model (PDPM)

Patient-Driven Payment Model (PDPM) is the payment model for Medicare skilled nursing care reimbursement. The PDPM is a case-mix classification system that aims to classify patients into payment groups based on patient characteristics rather than service use. Patients are assigned to 1 group for each of the 5 case-mix adjusted components that determines the per-diem rate by case-mix index (CMI) and further assigned into a CMI resident group. Physical Therapy (PT), Occupational Therapy (OT), and Non-Therapy Ancillary (NTA) payments are adjusted to account for changes in resource use during the stay. These adjusted rates are added to the unadjusted SLP and nursing components and the non-case-mix component to determine the final per diem rate for the patient. The 5 case-mix adjusted components are(4):

1. Physical Therapy (PT) - 16 groups, based on PT being the primary reason for SNF care and functional status on admission. Table 1 shows the PT/OT Clinical Category, the functional score based on Section GG of MDS 3.0, and the corresponding case-mix index (CMI) group.

- 2. Occupational Therapy (OT) - 16 groups, based on OT being the primary reason for SNF care and functional status on admission. Table 1 shows the PT/OT Clinical Category, the functional score based on Section GG of MDS 3.0, and the corresponding case-mix index (CMI) group.
- 3. Speech-Language Pathology (SLP) - 12 groups, based on SLP being the primary reason for SNF care, cognitive status on admission, SLP-related comorbidities, and the presence of a swallowing disorder or mechanically altered diet. Table 2 shows the clinical category, diet alteration need, and the corresponding case-mix index (CMI) group
- 4. Non-Therapy Ancillary (NTA) - 6 groups, based on the presence of comorbidities and extensive services received. Table 3 shows the NTA score range based on number of comorbidities or need for extensive services and the corresponding case-mix index (CMI) group
- 5. Nursing - 25 groups, based on clinical information from the SNF stay. Table 4 shows the clinical condition, depression presence, restorative nursing care, functional score based on Section GG of MDS 3.0 and the corresponding case-mix index (CMI) group

PDPM does not include a therapy minutes component as a basis of payment. The Non-Therapy Ancillary (NTA) provides a separate CMI adjustment intended to accommodate medically complex patients requiring more items for care (eg, drugs, supplies). PDPM assessment schedule consists of 5-Day Scheduled, Interim Payment, and Discharge Assessments.(5) The patient's functional measure is determined by scoring of the patient on level of performance with activities of daily living (ADL) for eating, oral hygiene, toileting hygiene and mobility (includes transfers, sit-to-stand, ambulation) as coded in MDS 3.0 Section GG, **Functional Abilities and Goals**(4):

- Score 4 if coded 05 or 06: Set-up assistance, Independent
- Score 3 if coded 04: Supervision or touching assistance
- Score 2 if coded 03: Partial/moderate assistance
- Score 1 if coded 02: Substantial/maximal assistance
- Score 0 if coded 01, 07, 09, or 88: Dependent, Refused, N/A, Not Attempted

Table 1. Summary of PT and OT PDPM Clinical Categories and Case-Mix Index (CMI) Groups

Clinical Category	GG Based Function Score	Case-Mix Index (CMI) Group
Major joint replacement or spinal surgery	0-5	TA
Major joint replacement or spinal surgery	6-9	TB
Major joint replacement or spinal surgery	10-23	TC
Major joint replacement or spinal surgery	24	TD
Other orthopedic	0-5	TE
Other orthopedic	6-9	TF
Other orthopedic	10-23	TG
Other orthopedic	24	TH
Medical management	0-5	TI

Medical management	6-9	TJ
Medical management	10-23	TK
Medical management	24	TL
Non-orthopedic surgery and acute neurologic	0-5	TM
Non-orthopedic surgery and acute neurologic	6-9	TN
Non-orthopedic surgery and acute neurologic	10-23	TO
Non-orthopedic surgery and acute neurologic	24	TP

Table 2. Summary of SLP PDPM Clinical Categories and Case-Mix Index (CMI) Groups

Presence of Acute Neurologic Condition, SLP-Related Comorbidity, or Cognitive Impairment	Mechanically Altered Diet or Swallowing Disorder	SLP Case-Mix Index (CMI) Group
None	Neither	SA
None	Either	SB
None	Both	SC
Any one	Neither	SD
Any one	Either	SE

Any one	Both	SF
Any two	Neither	SG
Any two	Either	SH
Any two	Both	SI
All three	Neither	SJ
All three	Either	SK
All three	Both	SL

The **Non-Therapy Ancillary (NTA)** component of PDPM allocates points for certain diagnoses or anticipated extensive use of services or supplies, as follows:

- 8 points for diagnosis of HIV/AIDS per claims
- 7 points for high level parenteral IV feeding
- 5 points for special treatments or programs with IV medication
- 4 points for special treatments or programs for ventilator or respirator care
- 3 points for low level parenteral IV feeding or lung transplant status
- 2 points for **1 or more** of the following:
 - Special Treatments/Programs: Transfusion Post-admit Code
 - Major Organ Transplant Status, Except Lung
 - Active Diagnoses: Multiple Sclerosis Code
 - Opportunistic Infections
 - Active Diagnoses: Asthma COPD Chronic Lung Disease Code
 - Bone/Joint/Muscle Infections/Necrosis—Except Aseptic Necrosis of Bone
 - Chronic Myeloid Leukemia
 - Wound Infection Code
 - Active Diagnoses: Diabetes Mellitus (DM) Code

- 1 point for **1 or more** of the following:
 - Endocarditis
 - Immune Disorders
 - End-Stage Liver Disease
 - Other Foot Skin Problems: Diabetic Foot Ulcer Code
 - Narcolepsy and Cataplexy
 - Cystic Fibrosis
 - Special Treatments/Programs: Tracheostomy Care Post-admit Code
 - Active Diagnoses: Multi-Drug Resistant Organism (MDRO) Code
 - Special Treatments/Programs: Isolation Post-admit Code
 - Specified Hereditary Metabolic/Immune Disorders
 - Morbid Obesity
 - Special Treatments/Programs: Radiation Post-admit Code
 - Highest Stage of Unhealed Pressure Injury—Stage 4
 - Psoriatic Arthropathy and Systemic Sclerosis
 - Chronic Pancreatitis
 - Proliferative Diabetic Retinopathy and Vitreous Hemorrhage
 - Other Foot Skin Problems: Foot Infection Code, Other Open Lesion on Foot Code, Except Diabetic Foot Ulcer Code
 - Complications of Specified Implanted Device or Graft
 - Bladder and Bowel Appliances: Intermittent Catheterization
 - Inflammatory Bowel Disease
 - Aseptic Necrosis of Bone
 - Special Treatments/Programs: Suctioning Post-admit Code
 - Cardio-Respiratory Failure and Shock
 - Myelodysplastic Syndromes and Myelofibrosis
 - Systemic Lupus Erythematosus, Other Connective Tissue Disorders, and Inflammatory Spondylopathies
 - Diabetic Retinopathy—Except Proliferative Diabetic Retinopathy and Vitreous Hemorrhage
 - Nutritional Approaches While a Resident: Feeding Tube
 - Severe Skin Burn or Condition
 - Intractable Epilepsy
 - Active Diagnoses: Malnutrition Code
 - Disorders of Immunity—Except: RxCC97: Immune Disorders
 - Cirrhosis of Liver
 - Bladder and Bowel Appliances: Ostomy
 - Respiratory Arrest
 - Pulmonary Fibrosis and Other Chronic Lung Disorders

Table 3. Summary of Non-Therapy Ancillary (NTA) PDPM Score Ranges and Case-Mix Index (CMI) Groups

NTA Score Range	NTA Case-Mix Index (CMI) Group
12+	NA
9-11	NB
6-8	NC
3-5	ND

1-2

NE

0

NF

Table 4. Summary of Nursing PDPM Clinical Categories and Case-Mix Index (CMI) Groups

Nursing RUG-IV	Clinical Conditions	Depression	Number of Restorative Nursing Services	GG Based Function Score	Nursing Case-Mix Index (CMI) Group
ES3	Tracheostomy and ventilator			0-14	ES3
ES2	Tracheostomy or ventilator			0-14	ES2
ES1	Infection			0-14	ES1
HE2/HD2	Serious medical conditions (eg, comatose, septicemia, respiratory therapy)	Yes		0-5	HDE2
HE1/HD1	Serious medical conditions (eg, comatose, septicemia, respiratory therapy)	No		0-5	HDE1

HC2/HB2	Serious medical conditions (eg, comatose, septicemia, respiratory therapy)	Yes	6-14	HBC2
HC1/HB1	Serious medical conditions (eg, comatose, septicemia, respiratory therapy)	No	6-14	HBC1
LE2/LD2	Serious medical conditions (eg, radiation therapy or dialysis)	Yes	0-5	LDE2
LE1/LD1	Serious medical conditions (eg, radiation therapy or dialysis)	No	0-5	LDE1
LC2/LB2	Serious medical conditions (eg, radiation therapy or dialysis)	Yes	6-14	LBC2
LC1/LB1	Serious medical conditions (eg, radiation therapy or dialysis)	No	6-14	LBC1
CE2/CD2	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	Yes	0-5	CDE2

CE1/CD1	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	No		0-5	CDE1
CC2/CB2	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	Yes		6-14	CBC2
CA2	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	Yes		15-16	CA2
CC1/CB1	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	No		6-14	CBC1
CA1	Conditions requiring complex medical care (eg, pneumonia, surgical wounds, burns)	No		15-16	CA1
BB2/BA2	Behavioral or cognitive symptoms		2 or more	11-16	BAB2
BB1/BA1	Behavioral or cognitive symptoms		0-1	11-16	BAB1

PE2/PD2	Assistance with daily living and general supervision	2 or more	0-5	PDE2
PE1/PD1	Assistance with daily living and general supervision	0-1	0-5	PDE1
PC2/PB2	Assistance with daily living and general supervision	2 or more	6-14	PBC2
PA2	Assistance with daily living and general supervision	2 or more	15-16	PA2
PC1/PB1	Assistance with daily living and general supervision	0-1	6-14	PBC1
PA1	Assistance with daily living and general supervision	0-1	15-16	PA1

Additional Levels of Care

Inpatient Rehabilitation

Inpatient rehabilitation facilities (IRF) payment is also based on HIPPS codes. The measurement tool in IRF settings is the Patient Assessment Instrument (PAI). This measurement tool classifies and assigns the patient to a Case-Mix Group (CMG) based on rehabilitation impairment categories, motor and cognitive functional status, and age, with additional CMGs based on short-stay status or death during IRF stay.(6)

End-of-Life

End-of-life care provides supportive care for terminally ill patients and their families. It may be provided in an inpatient setting (eg, hospital, freestanding facility, nursing home) or in the patient's home. Services may include nursing care, physicians' services, nurse practitioner services, medical social services, short-term inpatient care, religious or spiritual support, or volunteer assistance. Inpatient care is generally to provide pain control and symptom management or respite care for relief of the patient's caregivers. In some cases, a brief period of inpatient services may follow an acute hospital stay, while preparation is being made for the patient to receive end-of-life services at home.(7)

Patients who may benefit from end of-life-care include those who:

- Are terminally ill (ie, life expectancy is 6 months or less if illness runs its expected course)
- Require services for management of terminal illness and related conditions

Palliative Care

Palliative care is an approach that optimizes the quality of life of patients and their families facing the problems associated with any diagnosis or stage of illness. It begins at the point where life-prolonging treatment and chronic comfort care meet, offering a support system to help patients live as actively as possible.⁽⁷⁾⁽⁸⁾

Patients who may benefit from palliative care include those who:

- Desire symptom management with or without curative effect
- Need medical treatments
- Need a coping support system
- Guide transition through changing goals of care

Custodial Care

Custodial care is care that does not qualify as skilled care. In the PDPM classification, custodial care corresponds to PDPM categories Behavioral Symptoms and Cognitive Performance and Reduced Physical Function, which include the lowest 8 PDPM levels (ie, BAB2, BAB1, PDE2, PDE1, PBC2, PA2, PBC1, PA1). Generally speaking, these are patients who have needs for personal care services that do not require the continuing attention of trained medical or paramedical personnel, exceed the capabilities of the home physical environment and available caregiver services, and whose rehabilitation potential is limited. Their deficits fall into three general categories:

- Impaired cognition, including^{[B](3)}:
 - Comatose
 - Short-term memory problem
 - Cognitive skills for decision- making problem
 - Problem not being understood
 - Eating ADL (not self-performance)
- Behavior symptoms, including⁽³⁾:
 - Delusions
 - Hallucinations
 - Paranoia
 - Wandering
 - Care rejection
 - Agitation
 - Verbal or physical abuse
 - Inappropriate behavior
 - Disturbed sleep pattern
 - Risk of accidental self harm or neglect
- Reduced physical functioning, including⁽³⁾:
 - ADL score greater than 10
 - Patient does not meet conditions of any other PDPM category.^[C]

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Footnotes

[A] An active diagnosis is one that has a direct relationship to the patient's functional and cognitive status, mood or behavior, medical treatments, nursing monitoring, or risk of death during the 7-day look-back period.(3) [A in Context Link 1]

[B] Impaired cognition is demonstrated by a Staff Assessment for Mental Status score greater than 2. [B in Context Link 1]

[C] This includes patients who would meet the criteria for impaired cognition or behavior symptoms but have an ADL score greater than 10. [C in Context Link 1]

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